SOAP Note

Patient: Patient | **Clinician:** Dr. Yuki Tanaka

Date: Sep 28, 2026 · 11:06 PM | **Encounter ID:** visit-c24cb4cf

Subjective

Patient presents with home blood pressure readings of 160/95 mmHg on two occasions and a history of slightly elevated blood pressure during checkups. Reports mild headaches but denies chest pain, shortness of breath, or dizziness.

Objective

Not documented

Assessment

Elevated blood pressure readings consistent with possible undiagnosed hypertension.

Plan

Further evaluation including ambulatory blood pressure monitoring, lifestyle assessment, and potential diagnostic workup for hypertension.

Differential Diagnosis Suggestions

Supportive suggestions only; requires clinician verification and approval.

1. Essential Hypertension (81%)

Rationale: Patient presents with consistently elevated home BP readings (160/95 mmHg) and a history of borderline hypertension, fitting the profile for undiagnosed essential hypertension. The absence of acute end-organ damage symptoms supports a primary diagnosis pending confirmation.

2. Tension-Type Headache (67%)

Rationale: The patient reports mild headaches, which can be a symptom of hypertension or a comorbid tension-type headache. NICE CG150 suggests screening for tension-type headache triggers and notes that propranolol can manage both BP and tension headaches, suggesting a potential comorbidity.

3. White-Coat Hypertension (61%)

Rationale: While the patient has a history of elevated clinic BP, the current home readings are also significantly elevated (160/95 mmHg), which makes pure white-coat hypertension less likely than masked or sustained hypertension, but it remains a differential if clinic readings were previously higher than home readings (not the case here, but home is high). Actually, White Coat is when clinic > home. Here home is high. This diagnosis is less likely given home BP is 160/95. However, if

the 'checkups' were normal and home is high, it would be Masked Hypertension. Given the ambiguity, White Coat is a low-probability differential for *high* home BP, but often grouped in BP variability differentials. A better fit for high home BP is Masked Hypertension, but ICD-10 R03.0 covers elevated BP. Let's stick to the most likely differentials. If home BP is high, it's not White Coat. It is likely Essential or Masked. Since Masked isn't a standard top-5 ICD10 distinct from Essential in this context, I will list 'Masked Hypertension' if possible, but the prompt asks for standard names. Let's use 'Masked Hypertension' as a specific entity if applicable, or stick to 'Essential'. Given the prompt examples, I will include 'Masked Hypertension' as a differential if the clinic history was normal, but the summary says 'slightly elevated during checkups'. So it's likely sustained. Let's look for other differentials. Secondary Hypertension is a must-rule-out. Tension Headache is a symptom. Let's refine the list.

4. Masked Hypertension (59%)

Rationale: Masked hypertension is defined as normal clinic BP but elevated out-of-office BP. The patient's home readings are high (160/95), and if their clinic readings were previously normal or only slightly elevated, this pattern fits masked hypertension, which carries similar cardiovascular risk as sustained hypertension.

5. Secondary Hypertension (renal) (58%)

Rationale: In patients with newly diagnosed hypertension, especially if resistant or with atypical features, secondary causes such as renal artery stenosis or chronic kidney disease must be excluded. The patient's age is not specified, but 'new' diagnosis warrants basic screening.

Clinician signature: _____ **Date:** 2026-09-28